



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : ايمان محمد محمد رمضان
المحافظة : الجيزة
الحى / المدينة : العمرانية
العنوان : 0
رقم البطاقة : 27903120103964 رقم التليفون : 01006501720
تاريخ تسجيل الإستمارة : 30-09-2026
العمر (تاريخ الميلاد) : 12-03-1979 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي تراستوزوماب تحت الجلد و
بيرتوزوماب (فيسجو) [14117]

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس
وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المدونة في هذا النموذج صحيحة.
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / / د. / / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: إيمان محمد محمد رمضان
National ID: 27903120103964



تقرير طبي

المسيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الطول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي (Phesgo (Subcutaneous trastuzumab and pertuzumab

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



١١.٧٣/٣٢٢٨
١-٦٥٠١٧٢



atient Name	أيمان محمد محمد رمضان 0	Code Number	YK 275939
ix/Age	Female 47Y	Patient ID	20628
egistered Date	11/06/2026 08:38:56	Referred By Dr.	Out Patient
alidation Date	04/07/2026 14:11:51	Branch	Gustave Roussy
eporting Date	7/4/2026, 4:23:56 PM		

breast slide revision

Breast Slide Revision

Clinical Data: Right breast 9 O'clock mass, BIRADs 5.

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

Gross Examination:

- One referred block labelled by (1508-26). Together with, four referred immunostained slides for (ER, PR, Her2 & SISH).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Necrosis: Focal.

IPT:

- ER: Moderate nuclear reaction in 90% of tumor cells (score 7/8).
- PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified with wide small clusters.

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.
- IPT:
- ER: Positive (7/8).
- PR: Positive (5/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with wide small clusters.

Name: إيمان محمد محمد رمضان		
MR. : 11435		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 47-year-old pre-menopausal individual with a negative family history and no documented co-morbidities. The clinical history includes a diagnosis of invasive ductal carcinoma, grade 3. Core biopsy demonstrated invasive ductal carcinoma, grade 3, with estrogen receptor positivity 7/8, progesterone receptor positivity 5/8, and HER2 score +2 with amplification. Imaging revealed an ACR B sonomammographic pattern. On the right side, a 9-10 o'clock upper outer quadrant mass measuring 5 × 2 cm was noted, extending to the nipple, with retroareolar nodules causing nipple retraction on examination and a single level I pathological lymph node. The left breast showed mild fibrocystic changes corresponding to BIRADS 2. Staging workup with CT of the chest, abdomen, and pelvis and a bone scan were both negative for metastatic disease. The referring team recommended clips placement, neoadjuvant chemotherapy (NACT), followed by radiotherapy and central quadrantectomy with lymph node surgery according to response. The multidisciplinary team agreed on clips placement, insertion of a port-a-cath, administration of NACT with TCHP for six cycles, and subsequent radiotherapy with central quadrantectomy and lymph node surgery based on treatment response.		
تحريراً في ,		
	التاريخ :	23-09-2026
	الوقت :	
	الطبيب المعالج :	
يعتمد ,		

Patient Name	أيمان محمد محمد رمضان 0	Code Number	YK 275939
Sex/Age	Female 47Y	Patient ID	20628
Registered Date	11/06/2026 08:38:56	Referred By Dr.	Out Patient
Validation Date	04/07/2026 14:11:51	Branch	Gustave Roussy
Reporting Date	7/4/2026, 4:23:56 PM		

* **Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* **Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Prof. Dr. Gamal Abd El-Atty.
Dr. Walaa Ibrahim Ghonaim, MD.

Liver Function



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Misr El Qadima - Cairo, Egypt

العنوان : 987 كورنيش النيل مصر القديمة - رقم خدمة : 15580 / 0221277647

د. سلف
روسلو



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة العامة
للصحة

07/26/2026

MDT FORM

Name:	أحمد محمد أحمد	MRN:	11435
Age:	47 Y	ID:	27903120103964
Menopausal Sta	PRE	Phone:	01107313328
NRS SCORE:	PS 0	Marital Status:	MARRIED
BMI:	30	Family History:	-VE
Gender		Reason of MDT:	NEW CASE
CO Morbidity:	NAD	ECOC Performance Status	0 10 20 30 4
Surgical History:*	NORMAL DELIVERIES		

Clinical Data:

BX:

IDC G3

ER:

PR: 5/8

HER2: +2 (AMP) KI67:

History:

T N M

Sonomammography:

ACRB

RT 9-10 CLOCK UOQ MASS 5 X 2CM AND REACHING NIPPLE
RT RETROAREOLAR NODULES WITH NIPPLE RETRACTION BY EXAM

LT SINGLE LEVEL 1 PATHOLOGICAL LN

LT MILD FIBROCYSTIC CHANGES (BIRADS 2)

Biomarker/Molecular

BY EXAM MASSES AT ZONE A B

Mets Workup:

CT CAP " " " " FREE.

BONE SCAN " " " " " " " " FREE.

Recommendations:

FOR CLIPS BRACKTING, NACT ,THEN RT CENTRAL QUADRENTECTOMY
+ LNS ACCORDING RESPONSE

MDT Outcome & Plan

FOR CLIPS BRACKTING
PORTACATH INSERTION
NACT: TCHP X 6 cycles
THEN RT CENTRAL QUADRENTECTOMY + LNS ACCORDING TO RESPONSE

Clinical Staging:

Pathological Staging:

Q1 Right Side Q2 Q3 Q4 Q1 Left Side Q2 Q3 Q4

SIGNATURE

MEDICAL ONCOLOGY

SURGERY

RADIOLOGY

PATHOLOGY

2. Date of Report

2. Date of Report

about:blank

BQHEVA
Radiology Department

ب.ه.ة
قسم الأشعة الإشعاعية

PATIENT NAME: EMAN MOHAMMED MOHAMMED RAMADAN

GENDER: Female

DATE : 21-May-2026 10:48 AM

PATIENT ID: 10126280558

AGE: 47 y

ORDER: 3879846

99MTC-MDP BONE SCAN

Desired Information: -
skeletal survey.

Technique:

Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

No hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
Degenerative changes are seen over dorso-lumbar vertebrae.
Arthritic changes are seen over both knees.
Both kidneys are normally visualized.

Conclusion:

No definite evidence of metastatic osseous deposits.

Reported By,

Prof. Huda Fathy, Nuclear Medicine Consultant

21-May-2026 05:28 PM

PATIENT: EMAN MOHAMMED MOHAMMED RAMADAN, 10126280558, Female, 47 y

Report version 1, 21-May-2026 05:28 PM

Page 1 of 1



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16802

مركز ب.ه.ة
+ شارع حيدرة 101 شارع الميمون ج.د. الرياض
+ مستشفى ب.ه.ة التخصصي
+ الفاكس: 16802 البريد الإلكتروني: info@bqheva.org



PATIENT NAME: EMAN MOHAMMED MOHAMMED RAMADAN

PATIENT ID: 10126280558

GENDER: Female

AGE: 47 y

DATE : 03-Jun-2026 12:10 PM

ORDER: 3879837,3879892

Multislice post contrast CT scan of the chest, abdomen and pelvis revealed:**TECHNIQUE:**

- Multislice helical CT acquisition with automatic IV low osmolar non-ionic contrast medium infusion 90 ml.

FINDINGS:**(I) Chest:**

- The right breast shows a nodular enhancing lesions, the largest is seen measuring about 22 x 21 mm in its maximum axial dimensions, with irregular margins and surrounding mild parenchymal edematous changes.
- Enlarged right axillary lymph node is seen in level I measuring about 17 x 16 cm in its maximum axial dimensions, as well as few prominent ones in level I and II.
- Aberrant right subclavian artery is noted (variant).
- Both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density and enhancement with no focal lesions.
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, both kidneys, aorta and IVC.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.

PATIENT: EMAN MOHAMMED MOHAMMED RAMADAN, 10126280558, Female, 47 y

Report version 2, 04-Jun-2026 05:51 AM

Page 1 of 2



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16602

مركز البهية الجديد
أ.ع.ع. علوية أول شارع الثورة جوار دارى، الحجرة
مستشفى جامعة الشيخ زايد
ط. الثاني الأول - الشيخ زايد أ.ع.ع.م كلية الهندسة



- Bulky uterus with anterior wall myometrial homogenous lesion measuring 25 mm likely Fibroid.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Small left ischial sclerotic lesion is seen, Bone island.
- Dorsolumbar spondylo-degenerative changes.

OPINION:

- Right breast malignant-looking enhancing nodular lesion with metastatic-looking right axillary lymphadenopathy.
- No CT evidence of pulmonary, mediastinal, hilar, hepatic, adrenal, abdominal nodal, pelvic nodal, or peritoneal metastatic disease detected on the current study.

Reported By,

Dr. Ahmed Abdel-Monem, MSc

Dr. Amr Farouk, MD 04-Jun-2026 05:51 AM

PATIENT: EMAN MOHAMMED MOHAMMED RAMADAN, 10126280558, Female, 47 y

Report version 2, 04-Jun-2026 05:51 AM

Page 2 of 2



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مركز بحية الهدى
4 شارع عطية أول شارع الهدى بحوار دارى، البحيرة
مستشفى بحية الشيخ زائد
٤ الحي الأول - الشيخ زايد أمام كلية الهندسة



PATIENT ID: 10126280558

AGE: 47 y

3850689, 3850690, 3853800

ULTRASOUND

- Screening.

- CC and MLO view.

- Complementary ultrasound

• Scattered fibrolandular densities (ACR B).

- Scattered microcalcifications.
- Right UOQ irregular spiculated high density lesions all area measuring about 5x2cm.
- This was confirmed by Complementary Ultrasound which showed right 9-10 o'clock irregular spiculated interconnected hypoechoic solid mass lesions measuring about 1.5x0.7cm and 1.9x1.7cm with adistance 1.2cm in between the lesions being 0.5cm from the overlying skin and reaching the nipple
- Left retroareolar 12 o'clock hypoechoic nodules measuring 0.5x0.6 and 0.7x0.5cm with no detected internal vascularity by color doppler study.
- Bilateral scattered anechoic cysts.
- No left solid mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right single pathological level I axillary lymph node measuring 1.8x0.6cm with cortical thickening 0.7cm.
- Left nonspecific axillary lymph nodes.

PINION:

- Right 9-10 o'clock malignant looking mass lesions with ipsilateral single level 1 pathological lymph node as described for histopathological correlation (BIRADS 5)
- Right 12 o'clock in determinant nodules
- Left mild fibrocystic changes (BIRADS2).

Page 1 of 3

Report version 1, 02-May-2026 02:13 PM



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PATIENT: EMAN MOHAMMED MOHAMMED RAMADAN, 10126280558, Female, 47 y
Report version 1, 02-May-2026 02:13 PM

Page 3 of 3



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مركز باهية
1 شارع الوحدة 101 شارع الوحدة حي دارى، الدمام
مستشفى جامعة الملك سعود
ع. الخ. 1011 الشق الثالث طابق الخامسة



Patient Name
Sex/Age
Registered Date
Referral Date
Reporting Date

0	اميان محمد محمد رمضان
Female 47Y	
11/06/2026 08:38:56	
04/07/2026 14:11:51	
7/4/2026, 4:23:56 PM	

Code Number
Patient ID
Referred By Dr.
Branch

YK 275939
20628
Out Patient
Gustave Roussy

breast slide revision

Clinical Data: Right breast 9 O'clock mass, BIRADs 5.

Breast Slide Revision

Nature of Specimen: Referred block & slides (Tru-cut biopsy).

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- One referred block labelled by (1508-26). Together with, four referred immunostained slides for (ER, PR, Her2 & SISH).

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- * Histological type: Invasive duct carcinoma.
- * Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- * Ductal carcinoma in situ: Absent.
- * Angioinvasion: Absent.
- * Perineural invasion: Absent.
- * Necrosis: Focal.

IPT:

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- PR: Moderate nuclear reaction in 30% of tumor cells (score 5/8).
- Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).
- SISH: Amplified with wide small clusters.

Diagnosis:

Right breast mass, Referred block & slides (Tru-cut biopsy):

- Invasive duct carcinoma, Grade 3.
- IPT:
- ER: Positive (7/8).
- PR: Positive (5/8).
- Her2: Equivocal (score 2+).
- SISH: Amplified with wide small clusters.

atient Name
x/Age
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0	ايوان محمد محمد رمضان
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7/4/2026, 4:23:56 PM	

Code Number
Patient ID
Referred By Dr.
Branch

YK 275939
20628
Out Patient
Gustave Roussy

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

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the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Prof. Dr. Gamal Abd El-Atty.

Dr. Walaa Ibrahim Ghonaim, MD.

Liver Function



غرفة رقم (4) - الدور الثالث - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة 3 - مصر
Room No. (4) - Third Floor Building No. 987 - Nile Corniche - Maser El Qadima - Cairo, Egypt

MRN:	10126280558	D.O.B:	12/03/1979
Patient Name:	EMAN MOHAMMED MOHAMMED RAMADAN	Sex:	Female
IC No.:	27903120103964		
Location:	External Clinic		

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2605030209	Collection Date	03/05/2026 02:31 PM
Accession No	SUR-26-1508	Received Date	03/05/2026 02:31 PM
Sample Type	Tissue	Doctor	DALIA MAHMOUD MOHAMED MAHMOUD

Clinical Information

Right breast 9 o'clock lesion, BIRADs 5, US guided biopsy.

Gross Picture

Multiple tissue cores, ranging in size from 1 cm to 1.5 cm long, Totally submitted.

Microscopic Picture

A case of invasive duct carcinoma ,grade 3. Focal tumor necrosis is seen. No DCIS component nor lymphovascular emboli were detected .Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, grade 3

Laterality

Right

Specimen type:

U/S guided Tru-cut biopsy.

Histological grade

Mitosis score (1-3):

2

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3



Histological grade (1-3):
3

Diagnosis

Right breast 9 o`clock lesion, US guided biopsy:

Invasive duct carcinoma, grade 3.

Comments

ER, PR & Her2 are ordered.

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Marwa Mahmoud Abd Elkader, FEBP, IFCAP.

Co-signed by:

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Bagi Abdallah

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

Validated on 09/05/2026 11:06 AM

Print Date:

06/06/2026 08:50 AM

Page Number: 2 of 2



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مركز باهريا
مختبر علمية أول شارع الهند بحري داري، القاهرة

مستشفى باهريا للشيخ زايد

202606060850AM

MRN: 10126280558
 Patient Name: EMAN MOHAMMED MOHAMMED RAMADAN
 IC No.: 27903120103964
 Location: External Clinic
 D.O.B: 12/03/1979
 Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id	2605050176	Collection Date	05/05/2026 12:05 PM
Accession No	IMM-26-1705	Received Date	05/05/2026 12:05 PM
Sample Type	Paraffin Block	Doctor	Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
1508-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for ER/PR immunostaining.

System Used :
Ventana Benchmark ULTRA autostainer.

Antibody:
Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%–100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong).
Total score from 0 to 8.

Comment:
- There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.



- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:
Right

Diagnosis :

ER: Positive, weak nuclear reaction in 80% of tumor cells [5/8].

PR: Positive, intermediate nuclear reaction in 25% of tumor cells [5/8].

Signature

Heba A Abdelbaky (MD)

Co-Signed by:

Marwa Mahmoud Abd Elkader, FEBP , IFCAP.

Reported and electronically endorsed by

Marwa Mahmoud Abdelkader Nada

Pathology lab director:
Dr. Hazem Abdelazim, MD, IFCAP

Validated on 03/06/2026 02:27 PM

Print Date:

06/06/2026 08:50 AM

Page Number: 2 of 2



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مركز بحرية البحر
أ شارع طلقة أول شارع البحر جسر طاري, البحيرة

مستشفى بحرية الشيخ زايد

MRN: 10126280558
Patient Name: EMAN MOHAMMED MOHAMMED RAMADAN
IC No.: 27903120103964
Location: External Clinic

D.O.B: 12/03/1979
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2605050177
Accession No IMM-26-1706
Sample Type Paraffin Block

Collection Date 05/05/2026 12:05 PM
Received Date 05/05/2026 12:05 PM
Doctor Mohamed Mostafa Mohamed Mostafa

Immunohistochemical Report

pathology number
1508-26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $> 10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $> 10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $> 10\%$ of tumor cells.

(Laterality) breast Cancer:
Right

Laterality :
Right

Her2 Result
Equivocal [score 2+]. Recommended For SISH



Signature

Heba A Abdelbaky (MD)

Reported and electronically endorsed by

heba ahmed Abdel Baki Abdallah

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

Validated on 09/05/2026 11:06 AM

Print Date:

06/06/2026 08:50 AM

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مركز بحية البحر
1 شارع طابية اول شارع البحر جدار داني, الجيزة

استشاري طب الباطن والسكر

MRN:	10126280558	D.O.B:	12/03/1979
Patient Name:	EMAN MOHAMMED MOHAMMED RAMADAN	Sex:	Female
IC No.:	27903120103964		
Location:	External Clinic		

MOLECULAR GENETICS

Sish

Specimen Id	2605090260	Collection Date	09/05/2026 11:52 AM
Accession No	SISH-26-167	Received Date	09/05/2026 11:52 AM
Sample Type	Tissue	Doctor	Mohamed Mostafa Mohamed Mostafa

Sish Report

pathology number
1508/26

Clinical Data
Rt. breast cancer.

Gross Picture :
Referred block submitted for HER 2 detection by silver in situ hybridization.

System Used :
Ventana Benchmark ULTRA autostainer.

Probe:
Ventana Inform HER2 Dual ISH DNA Probe Cocktail

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018 Twenty nuclei (each containing red (Chr17) and black (HER2) signals) should be enumerated. The final results for the HER2 status are reported based on the ratio formed by dividing the sum of HER2 signals for all 20 nuclei divided by the sum of Chromosome 17 signals for all 20 nuclei. The amplification status is defined as Amplified if the HER2/Chromosome 17 ratio ≥ 2.0 and the average Her2 gene copy number is ≥ 4.0 . It is Non-Amplified if the HER2/Chromosome 17 ratio < 2.0 with the Her2 gene copy number is < 4.0 .
If the HER2/Chr17 ratio is < 2 and the Her2 gene copy number is between 4.0 and 6.0, or, HER2/Chr17 ratio is ≥ 2 and the Her2 gene copy number is < 4 , or HER2/Chr17 ratio is < 2 and the Her2 gene copy number is ≥ 6.0 , an additional work should be done.

(Laterality) breast Cancer:
Right

Site 1

Laterality #1



Right

Diagnosis #1

Rt. breast Cancer:

Her-2 gene: Amplified (Her2/Chr17 =2.04 and average Her2 copy number is 5.1).

Final Result #1

Amplified

Site 2

Signature

Heba A Abdelbaky (MD)

Co-Signed by:

Dr. Marwa Mahmoud Abd Elkader, FEBP, IFCAP.

Reported and electronically endorsed by

Marwa Mahmoud Abdelkader Nada

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

Validated on 18/05/2026 04:38 PM

Print Date:

06/06/2026 08:50 AM

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